Coordinated Telehealth Case Report

Contoso Health | Ticket TH-2026-0907-01 | Tier 1 learner submission

Use rule: Document evidence, not assumptions. Use only the synthetic identifiers supplied in the exercise. Do not convert role-label substitution into a claim of HIPAA de-identification.

Case Header

Field	Learner Entry
Ticket reference	TH-2026-0907-01
Date/time window	Ticket opened 09:40 local time; scheduled visit start 10:00 local time (Ticket Snapshot). Tier 1 working window documented here: 09:40–09:51. Times after 09:40 are the Tier 1 working-log timestamps recorded for this simulation; the ticket itself supplies only 09:40 and 10:00.
Clinician role	The requesting nurse practitioner (NP) at the Site Bravo outpatient clinic.
Visit status at intake	Scheduled virtual follow-up not yet started; about 20 minutes to visit start at intake. Three reported symptoms: scheduled chart not opening normally, camera unavailable in the Teams pre-join screen, visit link not reaching the scheduled meeting (Requester Statement).
Final recommendation	**DELAY.** Endpoint, audio and video are ready, but meeting access and Epic chart access remain unresolved and are with their owning teams (WT-1, WT-2).

Component 1 | Epic Hyperspace Client-Side Triage

Required Field	Learner Entry
Observed symptom/exact signal	Requester Statement, sentence 1: "I can sign in, but the scheduled patient chart is not opening normally." Ticket E1: "Selecting the scheduled chart returns an access-related message."
Client-side vs. application/build determination	**Epic-side application access/build condition, not client-side.** The client launches, the session is authenticated and non-chart navigation opens (E1), so a client

Required Field	Learner Entry
	launch/session failure is not supported. The Epic-side access artifact shows the expected ambulatory chart-access assignment is not present (E3). Scope is one user; no multi-user impact or service advisory (E7).
Evidence cited	Ticket E1 (client launches; session authenticated; non-chart navigation opens; access-related message on the scheduled chart). Ticket E3 (expected ambulatory chart-access assignment not present). Ticket E7 (no correlated service-health advisory; no broad multi-user impact). Tier 1 Epic Procedure, Stage B and LC6 Case-Specific Decision Aid, rows 1 and 3. Epic Hyperspace Client-Side Function Reference, Chart access row.
Approved Tier 1 action attempted	Stage A and Stage B classification only: confirmed one-user scope, confirmed the client/session is functioning, and reviewed the identity prerequisite and the Epic-side access artifact separately. No client recovery action was attempted because no client-side fault is evidenced; the Function Reference says not to clear caches, reset sessions or reinstall without a supporting runbook step. No Epic security or build change was made (Ticket Support boundary; E1).
Outcome	**Escalated.** Chart access is unresolved at Tier 1 and needs a privileged Epic-side change.
Warm-transfer reference, if required	**WT-2** to the Epic Application / Analyst Team, sent 09:49.

Component 2 | HIM Access Validation

Required Field	Learner Entry
Deployment-specific access prerequisite checked	Membership of Microsoft Entra group EHR-AMBULATORY-NP. The organization-specific provisioning map identifies this group as a prerequisite for this deployment (E2), so the comparison with the Epic-side state is permitted here. Validation Sequence steps 1–2 also recorded: requester identified by role label and ticket reference; Epic sign-in

Required Field	Learner Entry
	and session succeed (E1).
Microsoft Entra state, if applicable	**Present.** The requesting NP is shown as a current member of EHR-AMBULATORY-NP (E2). This does not, by itself, prove Epic chart authorization (E2; HIM Reference, Technical rule).
Epic-side access evidence	**Missing.** The supplied mock Epic-side access artifact shows the expected ambulatory chart-access assignment is not present for this synthetic user (E3).
Do the two states align?	**No – mismatched.** Entra prerequisite present; Epic-side access assignment missing. The two states were validated independently and compared only because the provisioning map documents the relationship. Why the assignment is absent is not established by the supplied evidence and is left to the owning team.
Owning layer/receiving team	Epic application layer – **Epic Application / Analyst Team** (HIM Reference, Routing Logic row 2; Escalation Team Reference, Case routing row 1). Not routed to Identity Administrators: no Entra prerequisite, MFA, Conditional Access or risk problem is evidenced.
Warm-transfer reference, if required	**WT-2** (shared with Component 1 – one Epic-side condition, one handoff).

Component 3 | Endpoint Readiness Verification

Required Field	Learner Entry
Checklist used	EHR-connected workstation (Checklist A) and Telehealth endpoint (Checklist B). Productivity device (Checklist C) not applicable to this ticket.
Items passed	**Checklist A:** Managed-device identity – PASS (managed Windows workstation, online; E4). Compliance state – PASS (Intune: Compliant; E4). Network reachability – PASS on supplied evidence (device online, E4; Epic session authenticated, E1; no advisory, E7). Epic client/session – PASS (E1). **Checklist B:** Microphone/speaker – PASS (detected, E4; audio test PASS, E5). Camera – PASS after correction

Required Field	Learner Entry
	(hardware detected, E4; preview PASS, E5). OS/app permissions – PASS after correction (E4, E5). Teams client/browser path – PASS (the Teams pre-join screen opens; Requester Statement, sentence 2). Connectivity observation – no supplied evidence of local or site degradation (E7).
Items failed	**OS camera permission for Teams – FAIL at intake** (Off; E4). Corrected at Tier 1; now PASS. **Meeting access – FAIL, unresolved** (superseded join reference; E6). See Component 4. **Time/date – not established:** no time/date signal is supplied in the ticket, so no PASS is claimed.
Evidence cited	Ticket E4: "Intune compliance state: Compliant. Device is online. Camera hardware is detected. OS camera permission for Teams is Off. Microphone and speakers are detected." Ticket E5: "Video preview: FAIL before the approved local permission correction; PASS after correction." Intune Compliant is treated as a managed-device compliance signal only. It did not show that the camera worked: the device was Compliant while the camera permission was Off (Endpoint Readiness Checklists, Evidence rule).
Approved Tier 1 action	Applied the approved local permission correction: turned the OS camera permission for Teams On, then re-tested the video preview in the Teams pre-join screen (E5; Escalation Team Reference, Case routing row 2). No Intune policy or managed setting was changed.
Outcome/escalation	**Resolved at Tier 1 for the camera permission** – re-test signal: video preview PASS (E5). No endpoint warm transfer: the Escalation Team Reference states none is required after a successful re-test, and no persistent hardware, driver or managed restriction is evidenced. The meeting-access failure is a scheduling/join-path item, escalated under Component 4 (WT-1).

Component 4 | Telehealth Pre-Session Checks and Failure-Point Analysis

Required Field	Learner Entry
Audio	**PASS** – microphone and speakers detected (E4); audio test PASS (E5).
Video	**FAIL at intake, PASS after correction** – camera detected (E4); video preview failed before the approved camera-permission correction and passed after it (E5).
Device permissions	**FAIL at intake, PASS after correction** – OS camera permission for Teams was Off (E4); set On under the approved local procedure; no managed or security control blocked the change (E5).
Meeting access	**FAIL – unresolved.** The join reference in the original clinician message differs from the current join reference in the scheduling record, and the supplied course artifact marks the clinician's reference as superseded (E6). The clinician's link does not reach the scheduled meeting (Requester Statement, sentence 2). Join failure reproduced from the ticket details, not by a live join. No Teams error code is supplied, so none is recorded. Join reference values are withheld from this report and cited by evidence ID only.
Connectivity observation	No supplied evidence establishes a local network failure. No correlated Microsoft 365 service-health advisory and no broad multi-user impact (E7). Network health is not inferred from the audio/video results (E5).
Most supported failure-point category	**Scheduling access.** E6 states this is scheduling/join-path evidence, not a Teams tenant-policy finding. Ruled out on the evidence: user devices (corrected and re-tested, E5), local connectivity (E7), Teams configuration (no policy or tenant evidence; E6, E7).
Escalation owner	**Scheduling Administration** – WT-1, sent 09:47 (Escalation Team Reference, Case routing row 3). Not routed to M365 / Teams Administration or Network Engineering: the reference says not to route solely because the workflow uses Teams or because a join attempt failed. Not routed to the Epic Analyst for scheduling integration: a connector fault is not established.

Warm-Transfer Summaries

Two warm transfers were made. WT-1 to Scheduling Administration (09:47) and WT-2 to the Epic Application / Analyst Team (09:49). No endpoint, identity, network or M365 / Teams transfer was needed. Each summary uses role labels and the ticket reference only, and requests one action.

WT-1 | Subject line: Ticket TH-2026-0907-01 | Warm transfer to Scheduling Administration | Superseded visit link – meeting access FAIL

Ticket number	TH-2026-0907-01
Incident timestamp/time window	Handoff sent 09:47 local time. Ticket opened 09:40; scheduled visit start 10:00.
Synthetic session description	The requesting NP at Site Bravo cannot reach the scheduled virtual visit for the patient associated with this ticket from the visit link originally received.
Reported symptom	The visit link received does not take the requesting NP into the scheduled meeting.
Affected scope	One clinician, one scheduled virtual visit. No broad multi-user impact reported; no correlated Microsoft 365 service-health advisory (E7).
Most likely failure point	Scheduling access – join-path record / current-link delivery discrepancy (E6). Not a Teams tenant-policy finding.
Receiving ownership layer	**Scheduling Administration** (appointment/join-path record and current-link delivery).

Diagnostic evidence

Check/Surface	Observed State	Interpretation
Meeting access (E6)	FAIL. The clinician's join reference differs from the current join reference in the scheduling record; the clinician's reference is marked superseded.	Scheduling/join-path discrepancy; the clinician holds an out-of-date link.
Audio / video / device permissions (E4, E5)	Audio PASS. Video and camera permission PASS after the approved local correction.	User devices are not the reason the visit cannot be joined.
Service and connectivity context (E7)	No correlated service-health advisory; no multi-user impact; no local network failure evidenced.	Teams configuration and local connectivity are not supported as failure points.

Actions already taken	Ran the four pre-session checks and recorded results. Corrected the OS camera permission for Teams under the approved local procedure; video preview re-tested PASS. Compared the join reference in the clinician's message against the current scheduling record (E6).

Check/Surface	Observed State	Interpretation
Actions deliberately not taken	No tenant-policy modification. No network, managed endpoint-security or scheduling-integration configuration changes outside Tier 1 authority. The scheduling record and scheduling integration were not edited, and no replacement link was generated or sent by Tier 1.	
Escalation trigger	The fix requires action on the appointment/join-path record and delivery of the current link, which the ticket Support boundary places outside Tier 1.	
Requested next action (one)	**Verify the current join reference in the scheduling record for this ticket and re-deliver the current visit link to the requesting NP, then confirm on the ticket.**	
Session/service status at handoff	**Unable to proceed** – the clinician cannot join the scheduled visit; visit start is on hold (clinician advised to delay).	

Summary body as sent:

Ticket TH-2026-0907-01 | 09:47 | To: Scheduling Administration. The requesting NP at Site Bravo is unable to join the scheduled virtual visit for the patient associated with this ticket. Meeting access check: FAIL. The visit link in the original clinician message carries a join reference that the scheduling artifact marks as superseded; the scheduling record holds a different, current join reference (ticket evidence E6). Audio check: PASS. Video and device-permission checks: PASS after Tier 1 corrected the OS camera permission for Teams. No correlated Microsoft 365 service-health advisory and no multi-user impact (E7). Tier 1 has not changed the scheduling record or scheduling integration. Impact: visit start is on hold; the scheduled start is 10:00 today. Action required: verify the current join reference in the scheduling record and re-deliver the current visit link to the requesting NP, then confirm on the ticket.

Associated chat message: "Scheduling Administration – warm transfer on Ticket TH-2026-0907-01. The requesting NP at Site Bravo holds a superseded visit link; meeting access FAIL. Summary is on the ticket. Please re-deliver the current link and confirm. Staying on the ticket for questions."

Completion check: evidence is specific and observable, not a guessed root cause | receiving team owns the identified layer per the Escalation Team Reference | only synthetic exercise data and role labels are included | summary states what Tier 1 checked and the one action requested | no ETA invented.

WT-2 | Subject line: Ticket TH-2026-0907-01 | Warm transfer to Epic Application / Analyst Team | Chart-access assignment missing

Ticket number	TH-2026-0907-01
Incident timestamp/time window	Handoff sent 09:49 local time. Ticket opened 09:40; scheduled visit start 10:00.
Synthetic session description	The requesting NP at Site Bravo can sign in to Epic but cannot open the chart for the patient associated with this ticket ahead of the scheduled virtual visit.
Reported symptom	The scheduled chart does not open normally; selecting it returns an access-related message (E1).
Affected scope	One user. Other Epic navigation works for the same user on the same

	workstation. No multi-user impact reported (E7).
Most likely failure point	Epic-side application access/build: the expected ambulatory chart-access assignment is not present (E3). Why it is absent is not established.
Receiving ownership layer	**Epic Application / Analyst Team** (chart authorization / security configuration).

Diagnostic evidence

Check/Surface	Observed State	Interpretation
Epic client/session (E1)	Client launches; session authenticated; non-chart navigation opens; access-related message on the scheduled chart.	Client launch/session failure is not supported.
Entra prerequisite (E2)	The requesting NP is a current member of EHR-AMBULATORY-NP, the documented prerequisite group for this deployment.	Identity prerequisite satisfied; this does not prove Epic chart authorization.
Epic-side access artifact (E3)	Expected ambulatory chart-access assignment is not present for this synthetic user.	Entra and Epic states are mismatched; the gap is on the Epic side.

Actions already taken	Confirmed one-user scope and a functioning client/session (Stage A/B classification). Validated the Entra prerequisite and the Epic-side access artifact separately, then compared them. Checked service context: no correlated advisory (E7).
Actions deliberately not taken	No Epic security class, template, provider configuration or build change. No Entra group membership change. No cache clear, session reset or client reinstall – no client-side fault is evidenced.
Escalation trigger	A privileged Epic-side access change is required; Epic security/build is outside Tier 1 authority (Ticket Support boundary; Tier 1 Epic Procedure, Stage B).
Requested next action (one)	**Review the ambulatory chart-access assignment for the requesting NP's Epic user and apply the expected assignment if it is confirmed missing, then confirm on the ticket.**
Session/service status at handoff	**Unable to proceed** for pre-visit chart review – Epic sign-in and other navigation are usable; chart access is blocked.

Summary body as sent:

Ticket TH-2026-0907-01 | 09:49 | To: Epic Application / Analyst Team. The requesting NP at Site Bravo can sign in to Epic but cannot open the chart for the patient associated with this ticket. Epic client launch: PASS. Session authentication: PASS. Non-chart navigation: PASS. Chart access: FAIL with an access-related message (ticket evidence E1). Entra prerequisite: PASS, the requesting NP is a current member of EHR-AMBULATORY-NP (E2). Epic-side access artifact: the expected ambulatory chart-access assignment is not present (E3). One user affected; no correlated service-health advisory (E7). Tier 1 has made no Epic security, build or group-membership change. Impact: pre-visit chart review is blocked; the scheduled start is 10:00 today. Action required: review the ambulatory chart-access assignment for the requesting NP's Epic user and apply the expected assignment if it is confirmed missing, then confirm on the ticket.

Associated chat message: "Epic Analyst team – warm transfer on Ticket TH-2026-0907-01. The requesting NP at Site Bravo has the Entra prerequisite but the Epic chart-access assignment is not present; chart access FAIL. Summary is on the ticket. Please review the assignment and confirm. Staying on the ticket for questions."

Completion check: evidence is specific and observable, not a guessed root cause | receiving team owns the identified layer per the Escalation Team Reference | only synthetic exercise data and role labels are included | summary states what Tier 1 checked and the one action requested | no ETA invented.

Coordinated Clinician Communication

Required Field	Learner Entry
Recommendation	**DELAY** (short hold; not yet a reschedule).
Plain-language rationale	Devices and workstation are ready (Components 3 and 4: audio PASS, video and permissions PASS after correction, Intune Compliant). The visit cannot start because the clinician holds a superseded visit link (Component 4, E6), and pre-visit chart review is blocked because the Epic chart-access assignment is missing while the Entra prerequisite is present (Components 1 and 2, E2/E3).
Specialized teams engaged	**Scheduling Administration** – confirm and re-deliver the current visit link (WT-1). **Epic Application / Analyst Team** – review the missing ambulatory chart-access assignment (WT-2). Expected time to fix: not estimated. Neither receiving team has supplied one.
Clinician next action	Stay signed in at the workstation, do not use the link in the original message, and join only from the new link when it arrives.
Next status checkpoint	Tier 1 status update to the clinician at 10:00; delay-or-reschedule decision point at 10:10 if either item is still open. These are Tier 1 check-in times, not resolution

Required Field	Learner Entry
	estimates.

Message as sent to the clinician (no patient-linked data):

Ticket TH-2026-0907-01 | 09:51 | To: the requesting NP at Site Bravo

Decision: DELAY the start of your scheduled virtual visit. Please do not begin yet.

Why: Your audio, camera and workstation are ready – the camera permission was corrected and the video preview now works. Two items are still open: the visit link you received is an outdated one, and your Epic chart-access assignment is missing.

Who is working on it: Scheduling Administration is confirming and re-sending your current visit link. The Epic Analyst team is reviewing your chart-access assignment. Neither team has given a fix time yet, so I will not guess one.

Your next step: Stay signed in at your workstation and do not use the link in the original message. Join only from the new link when it arrives.

Next update: I will message you at 10:00 with the status from both teams. If either item is still open at 10:10, I will ask you to choose between a further short hold and rescheduling.

Case Summary

Required Field	Learner Entry
Final state of Component 1	**Escalated – open.** Client and session healthy; chart access blocked by an Epic-side access condition (E1, E3). WT-2 with the Epic Application / Analyst Team.
Final state of Component 2	**Escalated – open.** Entra prerequisite present (E2); Epic-side chart-access assignment missing (E3); states mismatched. Same handoff, WT-2.
Final state of Component 3	**Resolved at Tier 1.** Camera permission corrected and re-tested PASS (E4, E5). Workstation Compliant and online. Time/date not established from supplied evidence.
Final state of Component 4	**Three of four checks PASS; meeting access FAIL – open.** Failure point: scheduling access (E6). WT-1 with Scheduling Administration.
Open actions/owner	Scheduling Administration: verify the current join reference and re-deliver the current visit link (WT-1). Epic Application / Analyst Team: review and correct the ambulatory chart-access assignment (WT-2). Tier 1: status update to the clinician at 10:00; decision point at 10:10; re-test the exact failed workflows (chart

Required Field	Learner Entry
	open, join from current link) once each team confirms.
Privacy check	No real PHI included in the learner artifact. Clinician and patient are referred to by role label; the ticket reference is the traceability anchor. No names, dates of birth, record numbers or appointment identifiers appear. Join reference values from E6 are withheld and cited by evidence ID. Role-label substitution is a course privacy-minimization convention, not a claim of HIPAA de-identification.

Evidence Exhibit Log

Exhibit	Source/Evidence Pointer	What it Demonstrates	Privacy Check
Exhibit 1	Ticket, Requester Statement, sentences 1–3: "I can sign in, but the scheduled patient chart is not opening normally. My camera is not available in the Teams pre-join screen, and the visit link I received is not taking me into the scheduled meeting."	The three reported symptoms and the time pressure (about 20 minutes to visit start).	Role label only; no identifiers.
Exhibit 2	Ticket, Provided Evidence E1 | Epic client/session: "Epic client launches successfully; user session is authenticated; non-chart navigation opens. Selecting the scheduled chart returns an access-related message."	Client launch/session is healthy; the failure is specific to chart access (Component 1).	No chart content or patient identifier.
Exhibit 3	Ticket, Provided Evidence E2 | Identity prerequisite: Entra group EHR-AMBULATORY-NP is a documented prerequisite; "The requesting NP is shown as a current member."	Entra prerequisite present; architecture documents the relationship, so comparison is allowed (Component 2).	Group name and role label only.

Exhibit	Source/Evidence Pointer	What it Demonstrates	Privacy Check
Exhibit 4	Ticket, Provided Evidence E3 | Epic access artifact: "the expected ambulatory chart-access assignment is not present for this synthetic user."	Epic-side access state is missing; owning layer is the Epic application (Components 1 and 2; WT-2).	Synthetic user by role label.
Exhibit 5	Ticket, Provided Evidence E4 | Endpoint state: "Intune compliance state: Compliant. Device is online. Camera hardware is detected. OS camera permission for Teams is Off. Microphone and speakers are detected."	Managed-device readiness, and that Compliant did not mean the camera was usable (Component 3).	Device state only; no device or user identifier.
Exhibit 6	Ticket, Provided Evidence E5 | Telehealth audio/video: "Audio test: PASS. Video preview: FAIL before the approved local permission correction; PASS after correction."	Audio PASS; video and device-permission re-test signal after the Tier 1 correction (Components 3 and 4).	No identifiers.
Exhibit 7	Ticket, Provided Evidence E6 | Meeting-access path: the current scheduling record and the original clinician message hold different join references; the clinician's is marked superseded. "This is scheduling/join-path evidence, not a Teams tenant-policy finding."	Meeting access FAIL and the scheduling-access failure point (Component 4; WT-1).	Join reference values withheld; cited by evidence ID.
Exhibit 8	Ticket, Provided Evidence E7 | Service context: "No supplied Microsoft 365 service-health advisory is correlated to this case. No broad multi-user impact is reported."	One-user scope; no service or network failure point is supported (Components 1, 3, 4).	No identifiers.
Exhibit 9	Escalation Team Reference, Case routing	Receiving teams for WT-1 and WT-2, and that no	Reference material; no

Exhibit	Source/Evidence Pointer	What it Demonstrates	Privacy Check
	rows 1–3; HIM Access Validation Reference, Routing Logic row 2; Telehealth Pre-Session Check Procedure, Ticket Expected Evidence table.	endpoint transfer is needed after the successful camera re-test.	case data.

Submission Check

- Confirmed – all five required report sections are complete (Components 1–4 and the Coordinated Clinician Communication), plus Case Header, Case Summary and Evidence Exhibit Log.
- Confirmed – every determination cites a supplied signal (E1–E7 or a named reference row) or records that evidence is not yet established (time/date; cause of the missing assignment).
- Confirmed – warm-transfer summaries WT-1 (Scheduling Administration) and WT-2 (Epic Application / Analyst Team) are included for the two unresolved out-of-scope conditions.
- Confirmed – the clinician communication leads with the decision (DELAY) and gives one clear next action.
- Confirmed – no real PHI is present in the case report, warm-transfer subject lines, summary bodies, chat messages or the clinician communication.